

Pemetrexed + cisplatin

Lung - NCCP 00317 - version 7

Your medicines

Pemetrexed - Cisplatin

Your treatment schedule

Pemetrexed 500 mg/m² IV, followed by cisplatin 75 mg/m² IV with pre- and post-hydration.

A picture of side effects

Tell your team about new or worsening symptoms. Numbers link each symptom box to the body map.

Pemetrexed - marrow / mouth / skin / kidney

Cisplatin - kidney / hearing / nerves / nausea

1 Blood / infection

Pemetrexed / cisplatin

Low blood counts can cause infection, bruising or anaemia. Fever or sudden illness needs prompt oncology advice.

3 Hearing / tinnitus

Cisplatin

Report new ringing in the ears or hearing change; cisplatin-related hearing damage can persist.

5 Nausea / vomiting

Cisplatin

Cisplatin has high emetogenic potential. Take the prescribed anti-sickness medicines and call if you cannot keep fluids down.

7 Skin

Pemetrexed

Rash can occur; use the prescribed corticosteroid prophylaxis and report severe skin change.

2 Kidneys / hydration

Pemetrexed / cisplatin

Kidney function is important for both medicines. Cisplatin requires hydration and electrolyte monitoring.

4 Nerves

Cisplatin

New numbness, tingling or difficulty with fine movement/walking should be reported before the next dose.

6 Mouth / bowel

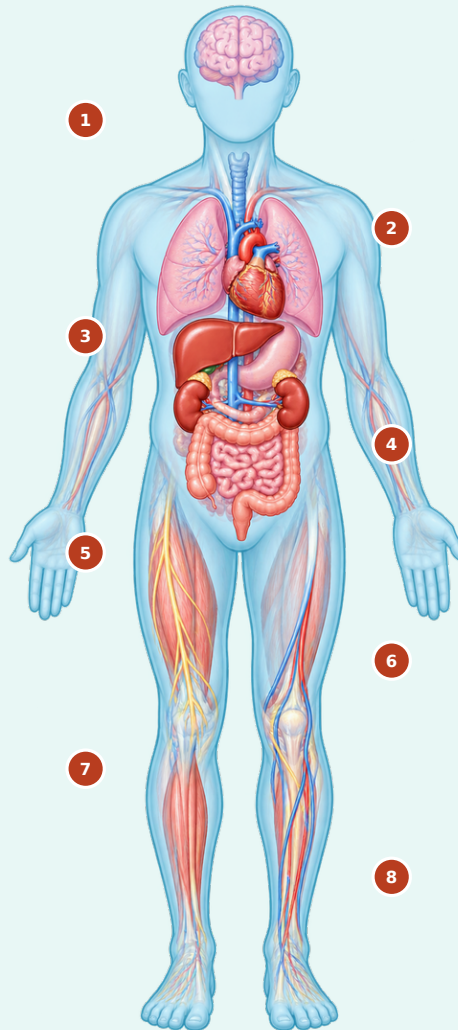
Pemetrexed

Mouth soreness or significant diarrhoea can occur and may require treatment review.

8 Electrolytes / whole-body symptoms

Cisplatin

Electrolytes can fall. Report marked weakness, cramps, palpitations or severe dizziness.



Selected effects only. The map does not show likelihood and cannot diagnose the cause of a symptom.

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Your treatment journey

Day 1: Pemetrexed 500 mg/m² IV, followed by cisplatin 75 mg/m² IV with pre- and post-hydration.

Every 21 days: Treatment repeats every 21 days.

Mesothelioma: Usually up to 4-6 cycles unless progression or unacceptable toxicity occurs.

NSCLC: Usually 4-6 cycles, then maintenance pemetrexed (NCCP 00222) where appropriate.

Monitoring and support

- Folic acid and vitamin B12 support are required with pemetrexed.
- Cisplatin requires hydration and electrolyte management according to local policy.
- Blood count, kidney and liver profile are checked before each cycle.
- Audiology may be considered where clinically indicated.
- Use the regimen-specific high-emetogenic-risk antiemetic prescription.

WHEN TO CALL

Contact oncology urgently for fever, inability to keep fluids down, marked reduction in urine, severe weakness, new hearing change, severe numbness/weakness, significant bleeding or rapidly worsening symptoms. For a medical emergency call 112/999.

Consent discussion support

Use the approved consent process to document treatment intent, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

<https://sactcheck.com/patient/00317/>

Official NCCP 00317 v7

SACTCheck v0.80.4 - patient information prototype for clinical review. Follow your oncology team and local emergency instructions.

